Cetuximab · every two weeks

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · antibody monotherapy

Active medicines: Cetuximab

Emetogenic risk: Low

Medicine	Dose / route	When
Chlorphenamine	10 mg intravenous (IV) bolus	At least one hour before cetuximab Repeat for subsequent cetuximab infusions; assess prior infusion reactions.
Dexamethasone	8 mg IV bolus	At least one hour before cetuximab on Day 1 This corticosteroid also covers the treatment-day corticosteroid part of antiemesis where appropriate. Pharmacy must reconcile route, dose and timing; do not automatically add a second 8 mg antiemetic dose.
Metoclopramide	10 mg PO up to three times daily as required	Rescue nausea treatment only as prescribed NCIS V8 rescue option; check contraindications, renal function, interactions and prescribed duration.

Additional phase-specific support

- Antiemetic classification refers to the active treatment day. Reconcile duplicate steroid entries before prescribing.
- No automatic granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) schedule is added; use the patient's neutropenia history, risk and consultant plan.
- Cetuximab premedication and infusion observation remain necessary even on an antibody-only day.
- Start local EGFR skin care; monitor magnesium, calcium and potassium before and periodically during therapy, then monthly for two months after the final dose.
- Review skin recovery, recurrence and the actual previous dose before any restart; no patient-led re-escalation.
- Observation attribution: NCCP permits stopping routine post-infusion observation after two tolerated doses; EMA Erbitux specifies close monitoring during and for at least one hour after infusion. Reconcile the approved local observation policy; do not automate removal of monitoring.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00732 v2 · p3 · NCCP NCIS antiemetics V8 (2025), Table 1 and multi-day principles

Sources checked 2026-11-05 · SACTCheck v0.84.35 · Regimen-level drafts; prescriber and oncology-pharmacy verification required